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EYE EXAMINATION FORM

Name of the Employee: pavan kalyan

Age: 24

Gender: Male— Female

Mobile Number: 7337010225

Date: 10/7/26

Employee ID: 22999

Referred by: Medi Assist

Chief Complaints:

c/o gue eye chp.

Refraction Details						
	UVA	SPHERE	CYL	AXIS	ADD	CVA
Right	6/6	—	—	—	—	6/6 N/6
Left	6/6	—	—	—	—	6/6 N/6

Colour Blindness: NAD

OPTOMETRIST

Signature of the Optometrist.

*Please note that the above details of power refraction is a part of the Basic Eye Examination. You are requested to visit any of the Speciality Eye hospital for detailed and final diagnosis.

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GE Healthcare REF I019728LSI

MR.PAVAN KALYAN MAHANTY
ID: 22999

Male

24 Years

10.07.2026 8:45:06
TENET DIAGNOSTICS
GACHIBOWLI
HYDERABAD

Location:
Room:
Order Number:
Indication:
Medication 1:
Medication 2:
Medication 3:

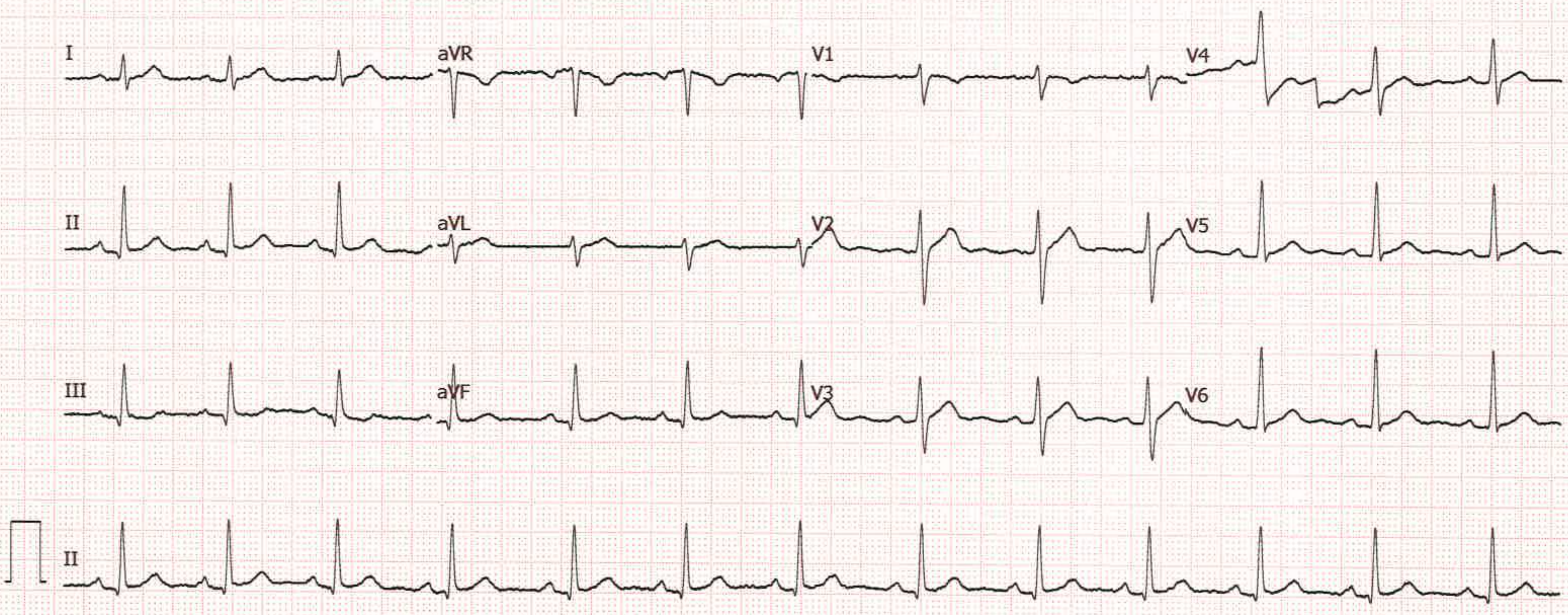
79 bpm
-- / -- mmHg

QRS :	84 ms
QT / QTcBaz :	358 / 410 ms
PR :	176 ms
P :	96 ms
RR / PP :	762 / 759 ms
P / QRS / T :	65 / 73 / 35 degrees

Normal sinus rhythm
Normal ECG

Dr. Y. Rohit Rao
MBBS
General Phys...

Technician:
Ordering Ph:
Referring Ph:
Attending Ph:



GE MAC2000 1.1 12SL™ v241 25 mm/s 10 mm/mV ADS 0.56-20 Hz 50 Hz 4x2.5x3 25 R1

Unconfirmed

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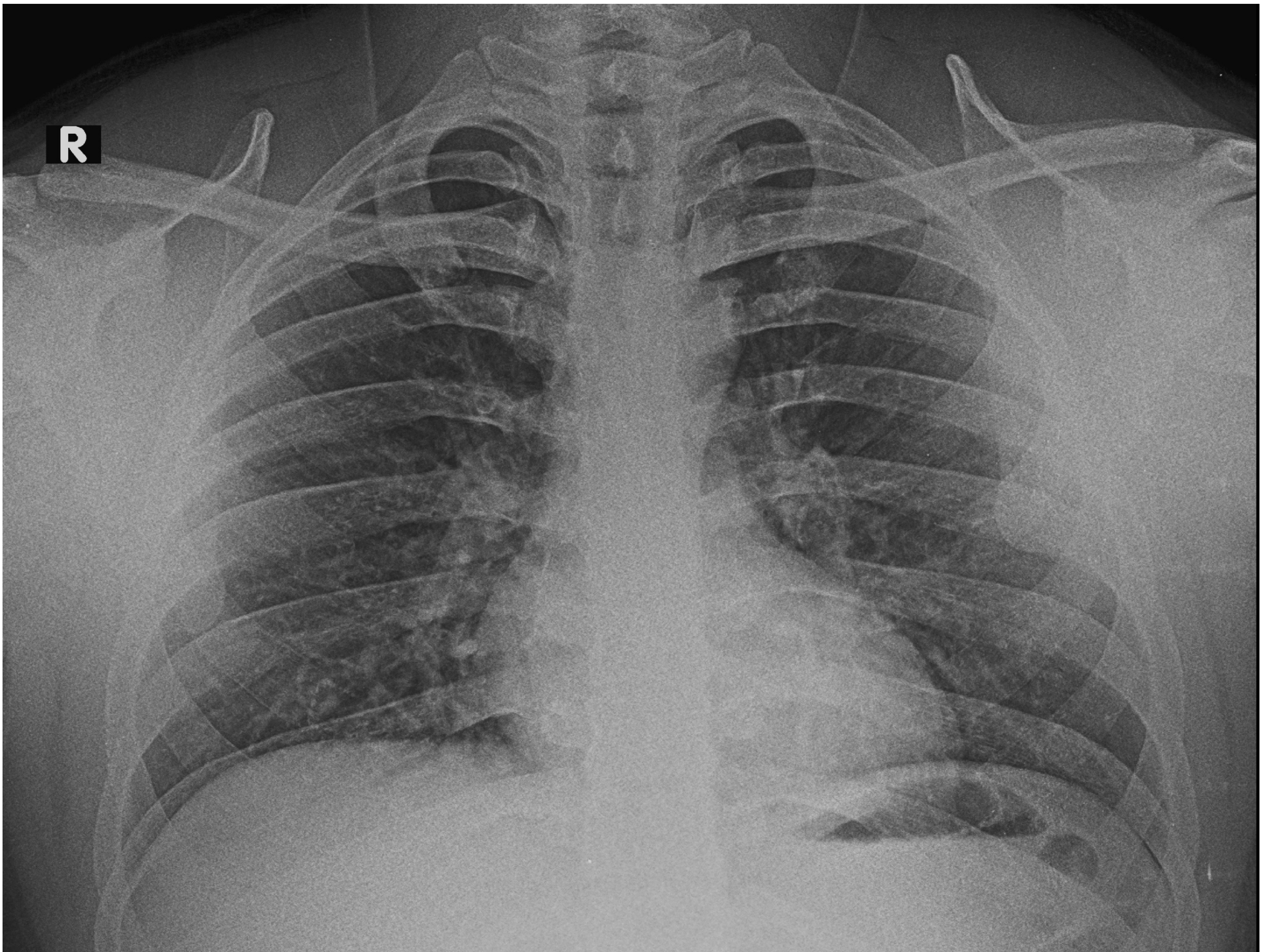

PAVAN KALYAN MAHANTY 59306555 26GCBH0022999 UMR4835207 40199525 CHEST PA 10-Jul-26
TENET DIAGNOSTICS GACHIBOWLI HYD

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Name	: **Mr . PAVAN KALYAN MAHANTY (59306555)**	TID	: UMR4835207
Age/Gender	: 24Y(s) /Male	Registered On	: 10-Jul-2026 07:46 AM
Ref By	:	Reported On	: 10-Jul-2026 09:40 AM
Reg.No	: 26GCBH0022999	Client Name	: Medi Assist

DEPARTMENT OF ULTRASOUND

Ultrasound Whole Abdomen

LIVER is normal shape, size (13.2 cms) and has uniform echopattern.
No evidence of focal lesion or intrahepatic biliary ductal dilatation.
Hepatic and portal vein radicals are normal.

GALL BLADDER shows normal shape. Evidence of calculus measuring 3 mm.
Gall bladder wall is of normal thickness.
CBD is of normal calibre.

PANCREAS has normal shape, size and uniform echopattern.
No evidence of ductal dilatation or calcification.

SPLEEN shows normal shape, size (8.5 cms) and echopattern.

KIDNEYS move well with respiration and have normal shape, size and echopattern. Cortico- medullary differentiations are well madeout.
No evidence of calculus or hydronephrosis.
Right kidney measures : 9.1 x 4.9 cms, Left kidney measures : 10.1 x 4.5 cms.

URINARY BLADDER shows normal shape and wall thickness.
It has clear contents. No evidence of diverticula.

PROSTATE shows normal shape, size and echopattern.
It measures : 3.1 x 2.9 x 3.4 cms, Vol : 16 cc.

No evidence of free fluid in the abdomen and pelvis.

IMPRESSION:

*** Small gall bladder calculus.**

Suggested clinical correlation and follow up

*** End Of Report ***

Typed By: Parveensulthana

Dr Seetha Devi
Consultant Radiologist

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Name	: **Mr . PAVAN KALYAN MAHANTY (59306555)**	TID	: UMR4835207
Age/Gender	: 24Y(s) /Male	Registered On	: 10-Jul-2026 07:46 AM
Ref By	:	Reported On	: 10-Jul-2026 11:41 AM
Reg.No	: 26GCBH0022999	Client Name	: Medi Assist

DEPARTMENT OF X-RAY

X-Ray Chest PA View

FINDINGS :

Lung fields appear normal.

Cardiac size is within normal limits.

Aorta and pulmonary vasculature is normal.

Bilateral domes of diaphragm and costophrenic angles are normal.

Visualised bones and soft tissues appear normal.

IMPRESSION:

*** Normal study.**

- Suggested clinical correlation and follow up.

*** End Of Report ***

Typed By: chamanthi

Dr Seetha Devi
Consultant Radiologist

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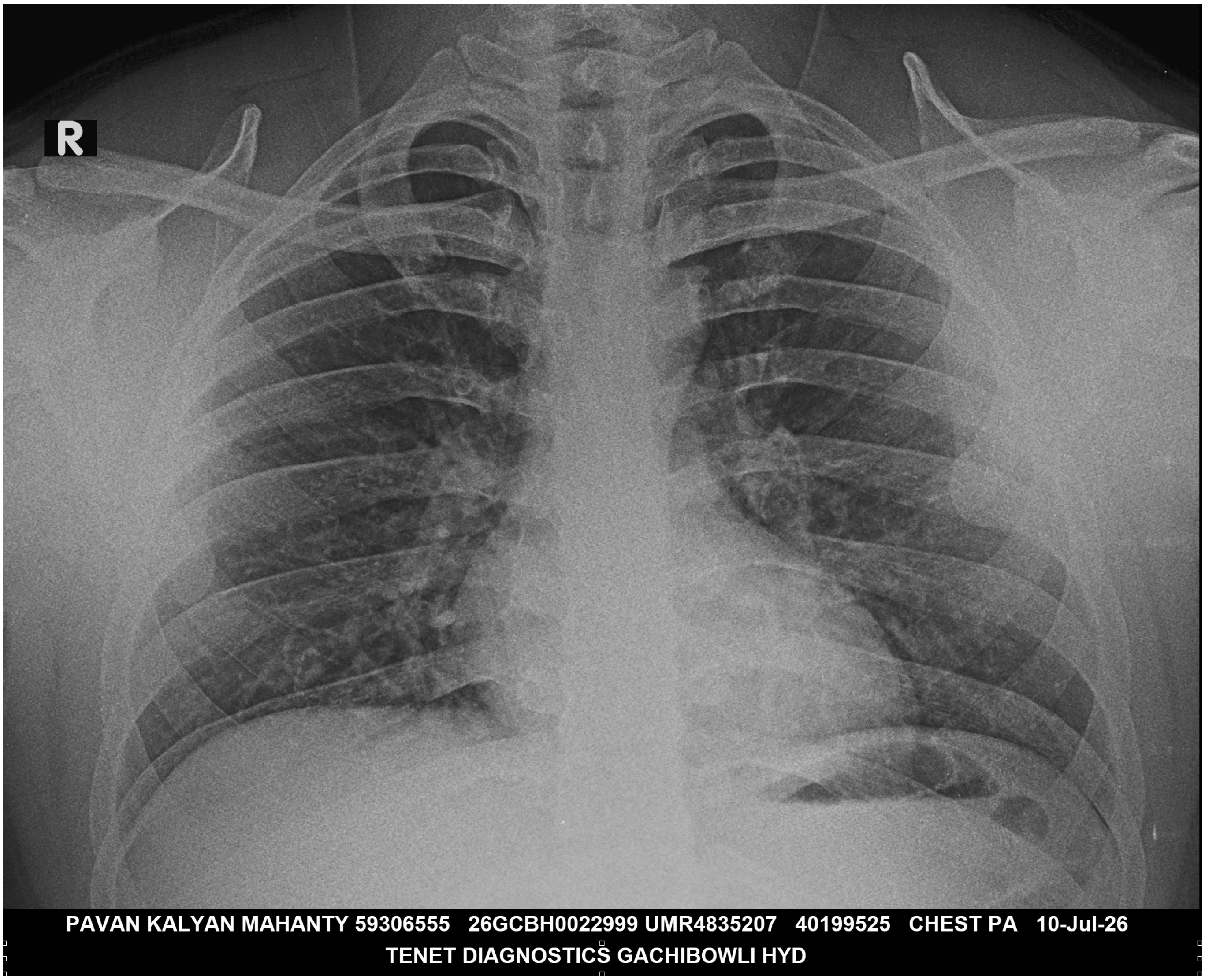
Ref By	:	Reported On	: 10-Jul-2026 11:41 AM
Reg.No	: 26GCBH0022999	Client Name	: Medi Assist



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TENET DIAGNOSTICS
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PLEASE SCAN QR CODE
TO VERIFY THE REPORT ONLINE

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226227
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY II

Thyroid Profile (T3,T4,TSH)

Investigation	Observed Value	Biological Reference Interval
Triiodothyronine Total (T3) Method:ECLIA	1.36	0.80-2.00 ng/mL
Thyroxine Total (T4) Method:ECLIA	7.7	5.1-14.1 µg/dL
Thyroid Stimulating Hormone (TSH) Method:ECLIA	2.79	0.27-4.20 µIU/mL

Interpretation:

A thyroid profile is used to evaluate thyroid function and/or help diagnose hypothyroidism and hyperthyroidism due to various thyroid disorders. T4 and T3 are hormones produced by the thyroid gland. They help control the rate at which the body uses energy, and are regulated by a feedback system. TSH from the pituitary gland stimulates the production and release of T4 (primarily) and T3 by the thyroid. Most of the T4 and T3 circulate in the blood bound to protein. A small percentage is free (not bound) and is the biologically active form of the hormones.
Reference: Tietz textbook of Clinial Chemistry and Molecular Diagnostics, Nader Rifia, Andrea Ritas Horvath, Carl T. Wittwer.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

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Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226225
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 13:20 PM
Sample Type	: Urine	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL PATHOLOGY

Complete Urine Examination (CUE)

Investigation	Result	Biological Reference Intervals
Physical Examination		
Colour Method:Physical	Pale Yellow	Straw to Yellow
Appearance Method:Physical	Clear	Clear
Chemical Examination		
Reaction and pH Method:Indicator	Acidic (5.5)	4.6-8.0
Specific gravity Method:Refractometry	1.020	1.000-1.035
Protein Method:Protein Error of pH indicators	Negative	Negative
Glucose Method:Glucose oxidase/Peroxidase	Negative	Negative
Blood Method:Peroxidase	Negative	Negative
Ketones Method:Sodium Nitroprusside Method	Negative	Negative
Bilirubin Method:Diazonium salt	Negative	Negative
Leucocytes Method:Esterase reaction	Negative	Negative
Nitrites Method:Modified Griess reaction	Negative	Negative
Urobilinogen Method:Diazonium salt	Negative	Up to 1.0 mg/dl (Negative)
Microscopic Examination		
Pus cells (leukocytes) Method:Flow Digital Imaging/Microscopy	1-2	2 - 3 /hpf
Epithelial cells Method:Flow Digital Imaging/Microscopy	1-2	2 - 5 /hpf
RBC (erythrocytes) Method:Flow Digital Imaging/Microscopy	Absent	Absent

Name	**: MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226225
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 13:20 PM
Sample Type	: Urine	Client Name	: Medi Assist

TEST REPORT

Casts Method:Flow Digital Imaging/Microscopy	Absent	Occasional hyaline casts may be seen
Crystals Method:Flow Digital Imaging/Microscopy	Absent	Phosphate, oxalate, or urate crystals may be seen
Others Method:Flow Digital Imaging/Microscopy	Nil	Nil

Method: Automated

Reference: Godkar Clinical Diagnosis and Management by Laboratory Methods, First South Asia edition. Product kit literature.

Interpretation:

The complete urinalysis provides a number of measurements which look for abnormalities in the urine. Abnormal results from this test can be indicative of a number of conditions including kidney disease, urinary tract infecation or elevated levels of substances which the body is trying to remove through the urine . A urinalysis test can help identify potential health problems even when a person is asymptomatic. All the abnormal results are to be correlated clinically.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Bandarupalli Divya
Consultant Pathologist
Reg.No - TSMC/FMR/10189

PLEASE SCAN QR CODE
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Name	: MR.PAVAN KALYAN MAHANTY (59306555)	TID/SID	: UMR4835207/ 32226224
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 16:18 PM
Sample Type	: Stool	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL PATHOLOGY

Stool Routine Examination

Investigation	Result	Biological Reference Interval
Macroscopic Examination		
Colour	Brown	
Consistency	Well formed	Well formed
Reaction & pH	Acidic (6.5)	5.0-8.0
Mucus	Absent	Absent
Blood Method:Peroxidase	Absent	Absent
Microscopic Examination		
Pus Cells	1-2	/hpf
Epithelial Cells	Occasional	/hpf
RBC (Erythrocytes)	Absent	Absent /hpf
Ova	Not found	Not found
Cysts	Not found	Not found
Trophozoites	Not found	Not found
Starch	Absent	
Vegetable Cells	Present	
Fat	Absent	Absent
Others	Nil	Nil

Method: Microscopy, Lugol's iodine.

Interpretation: Stool routine examination is an effective way to check for conditions which afffect the digestive system ,liver and pancreas.infections from bacteria,viruses or parasites,poor nutrient absorpton, and some forms of cancer can be deteted by stool examination. All abnormal results are to be correlated clinically.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Bandarupalli Divya
Consultant Pathologist
Reg.No - TSMC/FMR/10189

Name	: MR.PAVAN KALYAN MAHANTY (59306555)	TID/SID	: UMR4835207/ 32226223
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 16:11 PM
Sample Type	: Whole Blood	Client Name	: Medi Assist

TEST REPORT

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DEPARTMENT OF HEMATOPATHOLOGY

Blood Grouping ABO And Rh Typing

Parameter	Results
Blood Grouping (ABO)	A
Rh Typing (D)	Positive

Method:Microplate Automated Method

Interpretation :
The ABO grouping and Rh typing test determines blood type grouping (A,B, AB, O) and the Rh factor (positive or negative). A person's blood type is based on the presence or absence of certain antigens on the surface of their red blood cells and certain antibodies in the plasma. ABO antigens are poorly expresses at birth, increase gradually in strength and become fully expressed around 1 year of age.
In case of Rh(D) - Du(weak positive) or Weak D positive, the individual must be considered as Rh positive as donor and Rh negative as recipient.

Note :
Records of previous blood grouping/Rh typing not available. Please verify before transfusion.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Bandarupalli Divya
Consultant Pathologist
Reg.No - TSMC/FMR/10189

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226223
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 15:32 PM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 15:57 PM
Sample Type	: Whole Blood	Client Name	: Medi Assist

TEST REPORT

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DEPARTMENT OF HEMATOPATHOLOGY

Complete Blood Count (CBC) With ESR

Investigation	Result	Biological Reference Intervals
Hemoglobin Method:Spectrophotometry	15.6	13.0-17.0 g/dL
PCV/HCT Method:Calculated	45.7	40.0-50.0 vol%
Erythrocyte Count(RBC) Method:Electrical Impedance	5.26	4.50-5.50 mill /cu.mm
MCV Method:Calculated	86.9	83.0-101.0 fL
MCH Method:Calculated	29.8	27.0-32.0 pg
MCHC Method:Calculated	34.2	31.5-34.5 gm/dL
RDW (CV) Method:Calculated	14.0	11.6-14.0 %
Total WBC Count Method:Electrical Impedance	7880	4000-10000 cells/cumm
Platelet Count Method:Electrical Impedance	3.12	1.50-4.10 lakhs/cumm 10^3/µL
Neutrophils	56.3	40.0-80.0 %
Lymphocytes	32.7	20.0-40.0 %
Monocytes	9.4	2.0-10.0 %
Eosinophils	1.6	1.0-6.0 %
Basophils Method:Flow Cytometer - Microscopy	0.0	0.0-2.0 %
Absolute Neutrophil Count	4436.44	2000-7000 cells/cumm
Absolute Lymphocyte Count	2577	1000-3000 cells/cumm
Absolute Monocyte Count Method:Calculated	740.72	200-1000 cells/cumm 10^3/µL
Absolute Eosinophils Count	126.08	20-500 cells/cumm 10^3/µL
Absolute Basophil Count Method:Calculated	**0**	20-100 cells/cumm 10^3/µL

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226223
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 15:32 PM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 15:57 PM
Sample Type	: Whole Blood	Client Name	: Medi Assist

TEST REPORT

ESR 1st Hour Method:Westergren/Vesmatic	4	<=10 mm/hour

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Bandarupalli Divya
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Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226227
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Blood Urea Nitrogen (BUN)

Investigation	Observed Value	Biological Reference Interval
Blood Urea Nitrogen. Method:Calculated	11.4	6-20 mg/dL
Urea. Method:Urease	24.5	12.8-42.8 mg/dL

Interpretation: Urea is a waste product formed in the liver when protein is metabolized. Urea is released by the liver into the blood and is carried to the kidneys, where it is filtered out of the blood and released into the urine. Since this is a continuous process, there is usually a small but stable amount of urea nitrogen in the blood. However, when the kidneys cannot filter wastes out of the blood due to disease or damage, then the level of urea in the blood will rise. The blood urea nitrogen (BUN) evaluates kidney function in a wide range of circumstances, to diagnose kidney disease, and to monitor people with acute or chronic kidney dysfunction or failure. It also may be used to evaluate a person's general health status as well.

Reference: Tietz Fundamentals of Clinical Chemistry and Molecular Diagnostics

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226227
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

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DEPARTMENT OF CLINICAL CHEMISTRY I

Creatinine, Serum

Investigation	Observed Value	Biological Reference Interval
Creatinine. Method:Alkaline Picrate	1.03	0.70-1.20 mg/dL

Interpretation:

Creatinine is a nitrogenous waste product produced by muscles from creatine. Creatinine is majorly filtered from the blood by the kidneys and released into the urine, so serum creatinine levels are usually a good indicator of kidney function. Serum creatinine is more specific and more sensitive indicator of renal function as compared to BUN because it is produced from muscle at a constant rate and its level in blood is not affected by protein catabolism or other exogenous products. It is also not reabsorbed and very little is secreted by tubules making it a reliable marker. Serum creatinine levels are increased in pre renal, renal and post renal azotemia, active acromegaly and gigantism. Decreased serum creatinine levels are seen in pregnancy and increasing age.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

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Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

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Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226226F
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Sodium Fluoride Plasma	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Glucose Fasting (FBS)

Investigation	Observed Value	Biological Reference Interval
Glucose Fasting Method:Hexokinase	86	Normal: <100 mg/dL Impaired FG: 100-125 mg/dL Diabetes mellitus: >/=126 mg/dL

Interpretation: It measures the Glucose levels in the blood with a prior fasting of 9-12 hours. The test helps screen a symptomatic/ asymptomatic person who is at risk for Diabetes. It is also used for regular monitoring of glucose levels in people with Diabetes.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2025

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226226P
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 09:40 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 13:42 PM
Sample Type	: Sodium Fluoride Plasma	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Glucose Post Prandial (PPBS)

Investigation	Observed Value	Biological Reference Interval
Glucose Post Prandial Method:Hexokinase	**74**	Normal : <140 mg/dL Impaired PG: 140-199 mg/dL Diabetes mellitus: >/=200 mg/dL
Note	The discordant post prandial blood glucose values levels are observed in some of the conditions related to defective absorption, insufficient dietary intake, endocrine disorders, hypoglycemic drug overdose and reactive hypoglycemia etc.	

Interpretation: This test measures the blood sugar levels 2 hours after a normal meal. Abnormally high blood sugars 2 hours after a meal reflect that the body is not producing sufficient insulin which is indicative of Diabetes.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2025

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr.Abdur Rehman Asif
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Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Whole Blood	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Glycosylated Hemoglobin (HbA1C)

Investigation	Observed Value	Biological Reference Interval
Glycosylated Hemoglobin (HbA1c) Method:High-Performance Liquid Chromatography	5.2	Non-diabetic: <= 5.6 % Pre-diabetic: 5.7 - 6.4 % Diabetic: >= 6.5 %
Estimated Average Glucose (eAG) Method:Calculated	103	mg/dL %

Interpretation:

It is an index of long-term blood glucose concentrations and a measure of the risk for developing microvascular complications in patients with diabetes. Absolute risks of retinopathy and nephropathy are directly proportional to the mean HbA1c concentration. In persons without diabetes, HbA1c is directly related to risk of cardiovascular disease.

1) Low glycated haemoglobin (below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.

2) Interference of Hemoglobinopathies in HbA1c estimatiion:
A. For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
B. Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status
C. Heterozygous state detected (D10 is corrected for HbS and HbC trait).
3) In known diabetic patients, HbA1c can be considered as a tool for monitoring the glycemic control.
Excellent Control - 6 to 7 %,
Fair to Good Control - 7 to 8 %,
Unsatisfactory Control - 8 to 10 %
and Poor Control - More than 10 %.
Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2025.
* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226227
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Lipid Profile

Investigation	Observed Value	Biological Reference Interval
Total Cholesterol Method:Cholesterol Oxidase	176	Desirable: <200 mg/dL Borderline: 200-239 mg/dL High: >/=240 mg/dL
HDL Cholesterol Method:Direct Measurement	40	Low: <40 mg/dL High: >/=60 mg/dL
VLDL Cholesterol Method:Calculated	37.00	6.0-38.0 mg/dL
LDL Cholesterol Method:Calculated	99	Optimum: <100 mg/dL Near/above optimum: 100-129 mg/dL Borderline: 130-159 mg/dL High: 160-189 mg/dL Very high: >/=190 mg/dL
Triglycerides Method:Enzymatic end point	185	Normal:<150 mg/dL Borderline: 150-199 mg/dL High: 200-499 mg/dL Very high: >/=500 mg/dL
Chol/HDL Ratio Method:Calculated	4.40	Low Risk: 3.3-4.4 Average Risk: 4.5-7.1 Moderate Risk: 7.2-11.0
LDL Cholesterol/HDL Ratio Method:Calculated	2.48	Desirable: 0.5-3.0 Borderline Risk: 3.0-6.0 High Risk: >6.0
Non HDL Cholesterol Method:Calculated	**136**	<130 mg/dL

Interpretation: Lipids are fats and fat-like substances which are important constituents of cells and are rich sources of energy. A lipid profile typically includes total cholesterol, high density lipoproteins (HDL), low density lipoprotein (LDL), chylomicrons, triglycerides, very low density lipoproteins (VLDL), Cholesterol/HDL ratio .The lipid profile is used to assess the risk of developing a heart disease and to monitor its treatment. The results of the lipid profile are evaluated along with other known risk factors associated with heart disease to plan and monitor treatment. Treatment options require clinical correlation.

Reference: Third Report of the National Cholesterol Education program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III), JAMA 2001.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

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Dr.Abdur Rehman Asif
Consultant Biochemist
Reg.No - APMC/FMR/78102

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MC-2977

PLEASE SCAN QR CODE
TO VERIFY THE REPORT ONLINE

Name	: **MR.PAVAN KALYAN MAHANTY (59306555)**	TID/SID	: UMR4835207/ 32226227
Age / Gender	: 24Y(s) / Male	Registered on	: 10-Jul-2026 / 07:46 AM
Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Liver Function Test (LFT)

Investigation	Observed Value	Biological Reference Interval
Total Bilirubin. Method:Diazo Method	0.43	<1.2 mg/dL
Direct Bilirubin. Method:Diazo Method	0.18	<0.30 mg/dL
Indirect Bilirubin. Method:Calculated	0.25	<0.9 mg/dL
Alanine Aminotransferase ,(ALT/SGPT) Method:UV wtihout P5P	27	<50 U/L **Note:** Biological reference ranges have changed due to an update in kit generation.
Aspartate Aminotransferase,(AST/SGOT) Method:UV wtihout P5P	21	<50 U/L **Note:** Biological reference ranges have changed due to an update in kit generation.
ALP (Alkaline Phosphatase). Method:PNPP-AMP Buffer	78	40-129 U/L
Gamma GT. Method:GCNA	23	10-71 U/L
Total Protein. Method:Biuret	7.1	6.6-8.7 g/dL
Albumin. Method:Bromocresol Green (BCG)	4.5	3.5-5.2 g/dL
Globulin. Method:Calculated	2.60	1.8-3.8 g/dL
A/GRatio. Method:Calculated	1.73	0.8-2.0
AST/ALT Ratio Method:Calculated	0.78	<1.00

Interpretation: Liver functions tests help to identify liver disease, its severity, and its type. Generally these tests are performed in combination, are abnormal in liver disease, and the pattern of abnormality is indicative of the nature of liver disease. An isolated abnormality of a single liver function test usually means a non-hepatic cause. If several liver function tests are simultaneously abnormal, then hepatic etiology is likely.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

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Ref.By	: -	Collected on	:
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TEST REPORT

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Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Protein Total, Serum

Investigation	Observed value	Biological Reference Interval
Total Protein. Method:Biuret	7.1	6.6-8.7 g/dL

Interpretation: This test measures Protein levels in the blood. Proteins are an essential part of the tissues and cells that make up the body. This test measures the total amount of all proteins (including albumin and globulins) in blood. Protein measurements are a reflection of a number of health conditions including nutritional status, liver and kidney function.

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Ref.By	: -	Collected on	: 10-Jul-2026 / 07:50 AM
Req.No	: 26GCBH0022999	Reported on	: 10-Jul-2026 / 12:01 PM
Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Uric Acid, Serum

Investigation	Observed Value	Biological Reference Interval
Uric Acid. Method:Uricase	**7.2**	3.4-7.0 mg/dL
Note	Kindly correlate clinically	

Interpretation

It is the major product of purine catabolism. Hyperuricemia can result due to increased formation or decreased excretion of uric acid which can be due to several causes like metabolic disorders, psoriasis, tissue hypoxia, pre-eclampsia, alcohol, lead poisoning, acute or chronic kidney disease, etc. Hypouricemia may be seen in severe hepato cellular disease and defective renal tubular reabsorption of uric acid.

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Sample Type	: Serum	Client Name	: Medi Assist

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Bun/Creatinine Ratio

Investigation	Observed Value	Biological Reference Interval
BUN/Creatinine Ratio Method:Calculated	11.1	10-20

Interpretation:
The BUN/Creatinine ratio blood test is used to diagnose acute or chronic renal disease. BUN (blood urea nitrogen) and creatinine are both filtered in the kidneys and excreted in urine. The two together are used to measure overall kidney function

1. Increased ratio (>20) with normal creatinine occurs in the following conditions:
a) Increased BUN (prerenal azotemia), heart failure, salt depletion, dehydration
b) Catabolic states with tissue breakdown
c) GI hemorrhage
d) Impaired renal function plus excess protein intake, production, or tissue breakdown
2. Increased ratio (>20) with elevated creatinine occurs in the following conditions:
a) Obstruction of urinary tract
b) Prerenal azotemia with renal disease

3. Decreased ratio (<10) with decreased BUN occurs in the following conditions:
a) Acute tubular necrosis
b) Decreased urea synthesis as in severe liver disease or starvation
c) Repeated dialysis
d) SIADH
e) Pregnancy

4. Decreased ratio (<10) with increased creatinine occurs in the following conditions:
a) Phenacemide therapy (accelerates conversion of creatine to creatinine)
b) Rhabdomyolysis (releases muscle creatinine)
c) Muscular patients who develop renal failure

Reference source:

1. Brookes, E.M., Power, D.A. Elevated serum urea-to-creatinine ratio is associated with adverse inpatient clinical outcomes in non-end stage chronic kidney disease. Sci Rep 12, 20827 (2022).

2. Thomas L. Clinical Laboratory Diagnostics [Internet]. Prof. Lothar Thomas; 2024 [cited 2024 Jun 24]. https://www.clinical-laboratory-diagnostics.com/

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

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